



EBOLA PRESIDENTIAL FORCE TASK 17

Ministry of Public Health, Hygiene and Social Welfare

National Institute of Public Health

Public Health Emergency Operations Center

MVE17 Incident Management System



Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°121/MVEBDB/12/09/2026

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HIGHLIGHTS (last 24 hours)

At the end of the day on September 12, 2026, 87 new confirmed cases of Ebola Virus Disease (EVD), including 28 community deaths, were reported in the provinces of Ituri (44 cases), North Kivu (37 cases), Haut-Uélé (3), and Tshopo (3). In contrast, 10 deaths within Ebola Treatment Centers (ETCs) were reported. No new health zones have been affected in the last 24 hours.

ACCUMULATIVE CASE 7,200	ACCUMULATIVE DEATH CONFIRMED 3,475	LETHALITY 48.3%	PATIENTS IN ISOLATION/CTE 923	ACCUMULATIVE HEALED 1,712	TRACKING RATES CONTACTS (Of the Day) 88.0%
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Affected Provinces and Affected Health Zones



7 Provinces affected

Haut-Uélé, Ituri, North Kivu,
South Kivu, Tshopo, Bas-Uélé and
South Ubangi



62 out of 167
health zones affected
(37.1%)

Summary of key points from the last 24 hours

A total of **87 new confirmed cases** of Ebola Virus Disease (EVD), including **28 community deaths**, were reported by the end of the day on September 12, 2026. These confirmed cases are distributed across the provinces of **Ituri (44 cases)**, **North Kivu (37 cases)**, **Haut-Uélé (3 cases)**, and **Tshopo (3 cases)**, while those of Bas-Uélé and South Kivu recorded no new confirmed cases. Furthermore, **10 deaths within treatment centers** were recorded, including 6 in Ituri and 4 in North Kivu, bringing the total number of deaths for the day to **38**.

After obtaining two negative results for Ebola virus disease, **20 patients** were declared cured (**13 in Ituri, 5 in Haut-Uélé and 2 in Nord-Kivu**).

Since the start of the epidemic, the DRC has recorded **7,200 confirmed cases, including 3,475 deaths**, representing a case fatality rate of **48.3 %**. The epicenter remains the province of Ituri, which accounts for **78.0%** of the confirmed cases reported in the 7 affected provinces and also holds **(50.6 %)** of the new confirmed cases.

No new health zones have been affected in the last 24 hours.

DRC remains with **62 out of 167 health zones (37.1%)** affected and **7**

Affected provinces. Ituri has 28 out of 36 affected health zones, followed by North Kivu (16/34), Haut-Uélé (6/13), Tshopo (7/23), and South Kivu (1/34).

Bas-Uélé (3/11) and Sud Ubangi (1/16).

Table 1. Distribution of confirmed cases and deaths by affected province, as of September 12, 2026.

Province	New confirmed cases (24h)	Confirmed cases	Deaths (confirmed)	Lethality	Affected health zones
Ituri	44	5615	2,559	45.6%	28/36 (77.8%)
North Kivu	37	1269	786	61.9%	16/34 (47.0%)
Haut-Uélé	3	279	115	41.2%	6/13 (46.1%)
Tshopo	3	28	10	35.7%	7/23 (30.4%)
South Kivu	0	3	1	33.3%	1/34 (2.9%)
Bas-Uélé	0	5	3	60.0%	3/11 (27.3%)
South Ubangi	0	1	1	100.0%	1/16 (6.3%)
Total	87	7,200	3,475	48.3%	62/167 (37.1%)

Table 2. Distribution of confirmed cases and deaths by province and health zone, as of September 12, 2026.

Province / Health Zone	Cumulative number			Current situation (24 hours)			
	Case (n)	Deaths (n)	Lethality	New Case	Community deaths (Swab+)	Confirmed deaths within the CTE	Total confirmed deaths
Ituri	5615	2559	45.6%	44	20	6	26
Adja	11	1	9.1%				0
Aru	8	6	75.0%				0
Ariwara	8	2	25.0%				0
Aungba	12	5	41.7%				0
Bamboo	179	39	21.8%				0
Boga	2	2	100.0%				0
Bunia	1564	495	31.6%	14	8		8
Damascus	39	9	23.1%	3	0		0
Drodro	15	7	46.7%				0
Fataki	78	30	38.5%				0
Gethy	7	3	42.9%				0
Kambala	2	0	0.0%				0
Kilo	33	12	36.4%				0
Komanda	132	88	66.7%	5	4		4
Lita	259	127	49.0%	3	2		2
Logo	13	5	38.5%				0
Lolwa	31	8	25.8%				0
Mahagi	4	1	25.0%				0
Mambasa	17	7	41.2%				0
Mandima	47	25	53.2%	1	1		1
Mangala	312	146	46.8%	2	0		0
Mongbwalu	658	298	45.3%	4	2		2
Nia-Nia	237	120	50.6%	5	3		3
Nizi	729	273	37.4%				0
Nyankunde	125	36	28.8%				0
Rimba	11	4	36.4%				0

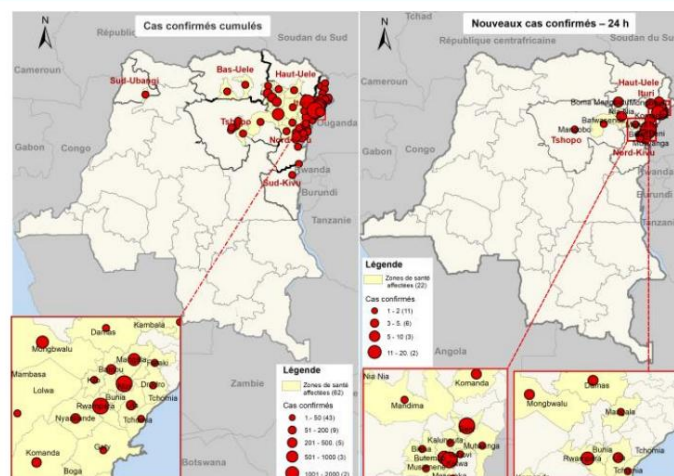
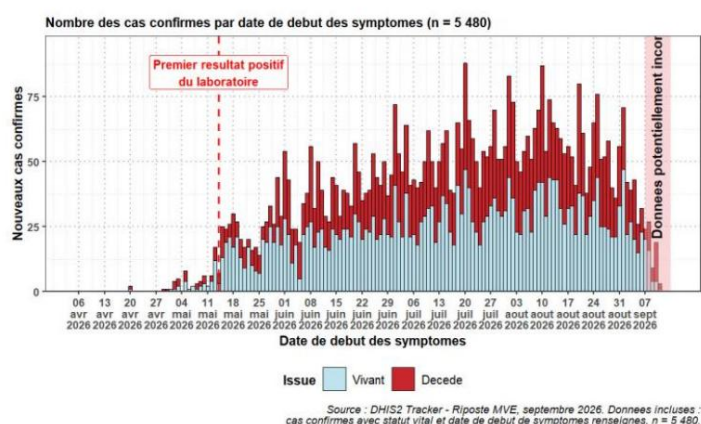
Rwampara	1013	349	34.5%	6	0		0
Tchomia	69	25	36.2%	1	0		0
To ventilate *	N / A	436	N / A			6	6
North Kivu	1269	786	61.9%	37	8	4	12
Blessed	222	162	73.0%	14	5	1	6
Good	16	7	43.8%	1	1		1
Butembo	236	152	64.4%	6	0		0
Goma	1	0	0.0%				0
Kalunguta	30	12	40.0%	1	0		0
Katwa	523	332	63.5%	10	1	1	2
Kayna	1	0	0.0%				
Kyondo	29	18	62.1%				0
Lubero	15	7	46.7%				0
Mabalako	19	9	47.4%				0
Manguredjipa	8	3	37.5%				0
Masereka	20	8	40.0%	1	0		0
Musienene	115	62	53.9%	2	1	2	3
Mutwanga	6	2	33.3%	1	0		0
Oicha	11	7	63.6%				0
Vuhovi	17	5	29.4%	1	0		0
Haut-Uélé	279	115	41.2%	3	0	0	0
Boma	32	14	43.8%	3	0	0	0
Mangbetu							
Gombari	3	1	33.3%				0
Isiro	96	41	42.7%				0
Pawa	57	25	43.9%				0
Rungu	2	1	50.0%				0
Wamba	89	33	37.1%				0
Tshopo	28	10	35.7%	3	0	0	0
Bafwasende	7	1	14.3%	2	0	0	0
Kabondo	4	1	25.0%				0
Lubunga	1	0	0.0%				0
Makiso-Kisangani	10	5	50.0%				0
Mangobo	4	2	50.0%	1	0	0	0
Tshopo	1	0	0.0%				0
Wanie-Rukula	1	1	100.0%				0
South Kivu	3	1	33.3%	0	0	0	0
Miti-Murhesa	3	1	33.3%				0
Bas-Uélé	5	3	60.0%	0	0	0	0
Buta	1	1	100.00%				0
Viadana	1	0	0.00%				0
Ganga	3	2	66.67%				0
South Ubangi	1	1	100.0%	0	0	0	0
Bulu	1	1	100.0%				0
Total	7200	3475	48.3%	87	28	10	38

*These are the confirmed deaths that occurred in the various CTEs in the province which are being broken down to be classified by health zone.

NA: Not Applicable.

Table 3. Status of alerts notified by province, as of September 12, 2026

Table 1: Status of alerts notified by provinces, as of September 11, 2018									
DPS	Alerts Received		Total alerts	Alerts verified and		Alerts verified and		Number of suspects investigated	Number of suspects investigated and transferred to CT/CTE (of the day)
	Living	Deceased		validated (suspected case)		invalidated (no suspected cases)			
				Living	Deceased	Living	Deceased		
Haut-Uélé	29	4	33	11	1	18	3	12	11
Bas-Uélé	31	0	31	6	0	25	0	6	3
Ituri	818	95	913	176	88	483	6	264	134
North Kivu	869	59	928	95	59	764	0	154	43
South Kivu	11	0	11	1	0	3	0	1	1
South Ubangi	ND	ND	ND	ND	ND	ND	ND	ND	ND
Tshopo	60	2	62	6	2	54	0	8	5
Total General	1818	160	1978	295	150	1,347	9	445	197

Figure 1. Evolution of cases by date of symptom onset over the last 21 days**Figure 2. Spatial distribution of confirmed cases (24h and cumulative) by health zone.**

1. COORDINATION

1.1. Main Actions

- Monitoring and support of response actions in all affected provinces.

2. RESPONSE ACTIONS BY PILLAR

1.1.1. Epidemiological surveillance

1.1.2. Main Actions

Of the seven affected provinces, six recorded a total of **1,978 alerts** on September 12, 2026, of which 1,801 (91.1%) were verified. Of these, **445 alerts were validated as suspected cases**. All of these were investigated (100%), and **197 were transferred to COVID-19 treatment centers** (Table 3).

Regarding the proportion of contact tracing, it stands at 91.3% (24,476/26,816), higher than both the previous day (87.5%) and the threshold of 85%; 88.7% in Ituri (13,230/14,920), 87.6% in North Kivu (10,780/12,310), 83.4% in Bas-Uélé (151/181), 82.0% in Tshopo (209/255) and 70.7% in Haut-Uélé (106/150).

1.1.3. Challenges

Contact tracing is suboptimal in 3 out of 5 provinces with ongoing contact tracing, with a significant decline in the Haut-Uélé province caused by low completeness.

1.2. Update on checkpoint and point of entry (PoE/PoC) activities

1.2.1. Main actions

Six (6) alerts were notified to the various PoE/PoC points on September 12th, five (5) in Ituri and one (1) in Haut-Uélé; all concerned deceased bodies, and no live alerts were validated. In Ituri, three bodies were intercepted at the Kasenyi PoC (Tchomia Health Zone) and two in the Mahagi Health Zone (Anzida PoC and DGDA checkpoint); two of these were swabs with negative results, while the other three passed through without swab documentation, representing a swab/secured proportion of 40% (2/5) of deceased bodies. In Haut-Uélé, one deceased body was intercepted at the PK10 Poko Road PoC (Isiro Health Zone), isolated, swab, and validated with a negative result (100%).

Table 4. Status of key PoE/PoC indicators by province, as of September 12, 2026

Indicators	Ituri	North Kivu	South Kivu	Tshopo	Haut-Uélé	Overall
Report completeness	96.7% (58/60)	100% (19/19)	55.5% (5/9)	100% (18/18)	54.5% (6/11)	90.6% (106/117)
% of functional strategic PoE/PoC	100%	100%	100%	100%	100%	100%
Number of people who have switched to PoE/PoC	135 245 89 127		8,361	16,105	12,090	260,928
% of travelers screened	98.00% 97.65% 98.80%	99.10% 98.00%				97.90%
% of travelers who washed their hands	97.20% 97.65% 98.80%	98.80% 97.00%				97.40%
% of travelers made aware: 99.40% 97.65% 100.00% 100.00% 100.00% 98.90%						
Number of alerts notified	5	0	0	0	1	6
Number of validated live alerts (suspected cases)	0	0	0	0	0	0
Number of bodies recovered today	5	0	0	0	1	6
% of suspected cases transferred to CT/CTE	N / A	N / A	N / A	N / A	N / A	N / A
% of lifeless bodies swabbed/secured	40% (2/5) N / A		N / A	N / A	100% (1/1)	50% (3/6)
% of suspected cases investigated within 24 hours	N / A	N / A	N / A	N / A	N / A	N / A

1.2.2. Challenges

Suboptimal operationalization of PoE/PoC coupled with low reporting completeness in Haut-Uélé (54.5%, 6/11) and Sud-Kivu (55.5%, 5/9); persistent insecurity hindering the commissioning of the Avakubi PoC (Ituri) and continuation of the strike for non-payment of the Nzebi PoC service providers; low encoding of alerts in DHIS2 Tracker and insufficient mobility for supervision in Rwampara and Bunia in Ituri; in Tshopo, 18 PoCs activated out of the 30 planned; in Sud-Kivu, absence of isolation device at the Kavimvira, Kalundu Port SNCC and Baraka PoEs; in Haut-Uélé, 5 PoE/PoCs (Dusu, Apodo, Aungba, Gossamu and Doko) did not report due to poor network connection.

1.3. Laboratory

1.3.1. Main actions

- **Ituri : 44 new positive results** (24 living and 20 dead) out of 248 new samples received and analyzed (**positivity: 17.7%**).
- **North Kivu: 37 new positive results** (29 living and 8 dead) out of 157 samples analyzed (**positivity of 23.5%**).
- **Haut-Uélé: 3 new positive results** (3 live) out of 25 samples analyzed (**positivity: 12%**).
- **Tshopo: 3 samples received and tested** (live), **all were found to be positive.**
- **Bas-Uélé: 3 samples** received and tested (2 live and 1 death), **all were found to be negatives.**

1.4. Infection Prevention and Control (IPC/EDS)

1.4.1. Main actions

- **In Ituri**, 28 of the 66 planned infection control zones (42.4%) were opened, 147 of the 220 identified households were decontaminated (66.8%), and 7 of the 9 health facilities (77.7%) were decontaminated; 60 infection prevention and control (IPC) kits were distributed to households in Mongbwalu. Regarding the health and safety surveys (HSS), 92 alerts were recorded, 74 HSS surveys were conducted (80.4%), and 63 bodies were swabbed. Nine failures were recorded, notably in Fataki and Mandima.
- **In North Kivu**, 31 of the 48 planned rings were opened (64.6%), 10 of the 66 identified households were decontaminated (15.2%), and 24 of the 36 health facilities were decontaminated (66.7%); the response rate and completeness were 62.5% (10/16 health zones reached). Regarding the blood and sanitation surveys (BSS), 61 alerts were received (including 21 reports from the previous day), 56 BSS were carried out, and 59 bodies were swabbed, with 2 failures in Kalunguta and Vuhovi.
- **In Tshopo**, the new Meteorological Treatment Center (CTE Météo) received an IPC (Infection Prevention and Control) assessment with a score of 90.3%; 15 community health centers (ESS) were monitored and supported; 148 frontline providers were briefed in 5 health zones; 12 local medical units (DLM) were monitored; 10 triage units were installed, and 500 liters of water were supplied to Lubunga; around the latest confirmed cases, a ring was opened in Bafwasende with decontamination of a health facility (FOSA) and 6 households, and an IPC/WASH kit was provided. Regarding the health data collection (EDS), 3 alerts were recorded and 3 bodies were swabbed (100%) for 2 EDSs carried out.
- **In Bas-Uélé**, 1 ring has been opened out of 1 expected (100%) in Ganga, with no households or health facilities decontaminated (0/1 for each). Decontamination of the household of the confirmed case from September 11 and of the Ganga General Referral Hospital is ongoing; briefing of 33 healthcare providers and 13 hygiene officers in 15 health establishments across 3 health zones
- **In Haut-Uélé**, 5 out of 6 planned rings were opened (83%), 5 out of 6 identified households were decontaminated (83%), and 1 out of 1 health facilities was decontaminated (100%), bringing the cumulative totals to 16/16 households and 8/8 health facilities. Regarding the health monitoring system (EDS), 2 alerts were recorded and 2 EDS were carried out with swab testing (100%).

1.4.2. Challenges

- **In Ituri** : low ring opening (42.4%; 28/66); insufficient decontamination teams in Nizi, Mambasa, Mangala and Komanda; lack of mobility in Mambasa, Bambu, Mandima and Mangala; insufficient PCI inputs in Mandima and Nizi; family resistance to decontamination in Mambasa; lack of EDS inputs and resistance in Bambu, Komanda, Nizi, Mambasa and Rwampara.
- **In North Kivu** : very low household decontamination rates (15.2%; 10/66) and still partial opening of the rings (64.6%; 31/48); need to activate, train, and motivate decontamination teams in the uncovered health zones (Katwa, Biena, Masereka, Kyondo, Mabalako, and Vuhovi); lack of traceability for some eligible household cases; lack of mobility for

the deployment of provincial PCI teams; need for training of PCI Data on collection tools; limited promptness and completeness (62.5%; 10/16 ZS affected).

- **In Tshopo** : insufficient IPC supplies in households, schools, markets, and churches; a low proportion of health establishments with an IPC score above 80% (2%; 2/91); and the Makiso-Kisangani General Referral Hospital incinerator remains out of service. In Bas-Uélé: lack of compliant waste management areas and sorting units in the 20 health establishment hotspots across 5 health zones; untrained IPC providers in the 5 priority health zones; and a lack of IPC supervisors.

1.5. Holistic care

1.5.1. Main actions

- **In Ituri**, there were 131 new admissions to the various dedicated facilities, compared to 88 discharges, including 13 recoveries. Thus, 476 patients are hospitalized out of 1015 beds, representing an occupancy rate of 46.9%; 435/463 (94%) patients received hot meals; 11/11 (100%) children are receiving LNPE/SLM at the CTE, 5/5 (100%) at the nursery, and 28/28 (100%) in the community.
- **In North Kivu**, 82 new admissions compared to 60 discharges recorded, including 2 recoveries; 342 patients are hospitalized, representing an overall occupancy rate of oversaturation (150.0%), with no beds available in the CTE/CT of Butembo and Katwa; 64/64 patients (100%), 8/8 accompanying persons and 52/52 frontline providers received hot meals.
- **In Haut-Uélé**, there were 11 new admissions and 9 discharges (5 recovered). Thus, 69 patients are hospitalized out of 134 beds, representing an occupancy rate of 51.5%; 42 patients, 55 accompanying persons and 26 PPLs received hot meals, with the exception of the Ebola Treatment Centers (ETCs) in Boma Mangbetu, Rungu and Gombari; prophylactic and specific treatment began at the Isiro ETC by the INRB team.
- **In Bas-Uélé**, 3 new admissions against 2 discharges (1 non-case and 1 escaped); 7 patients remain in isolation including 1 confirmed case taken care of at the HGR of Ganga, 11 high risk contacts are under prophylaxis in Buta and Ganga, for a completeness of the care data of 38% (5/13).
- **In Tshopo**, 9 new admissions were recorded compared to 3 discharges. Twelve (12) Patients are in isolation for 31 beds, representing an occupancy rate of 38.7%.
- **In South Kivu**, one (1) new admission was recorded, no discharges reported. Seventeen (17) patients are in isolation for 25 beds, representing an occupancy rate of 68.0%.

1.5.2. Challenges

Care still partially non-standard and limited capacities: **In Ituri**, the CTE/CTs are saturated on the side of the suspects: Nizi, Mambasa, Lolwa, Kilo, Mangala and Mandima.

In North Kivu, facilities are overcrowded (150.0%), with 49% of patients receiving care outside of treatment centers. **In Haut-Uélé**, the Pawa treatment center lacks hygiene facilities, and the Isiro center lacks space, water, and a constant supply of electricity. **In Tshopo**, the lack of safe blood and limited technical resources are hindering patient care. **In Bas-Uélé**, The absence of CTE/CT and ambulance limits the isolation of cases.

1.6. Vaccination

1.6.1. Main actions

- **In Ituri**, the training of vaccinators and sensitizers has ended as part of the preparations for the vaccination of PPL, which is scheduled to start on September 15, 2026 in Bunia.

1.6.2. Challenges

In Bas-Uélé, the stock shortage of ERVEBO in Buta is causing a temporary suspension of vaccination of PPL and TPL.

1.7. Continuity of Care

- In Tshopo, 119 cases received the experimental treatment, including 14 confirmed cases and 105 under post-exposure prophylaxis (68 treatments completed, 11 interrupted, and 26 ongoing); transfer of 3 high-risk contacts from the Ebola Treatment Center (ETC) to the transit center at Makiso-Kisangani General Referral Hospital (HGR Makiso-Kisangani), and briefing of CREC staff on the operation of the ETC/CT. In Haut-Uélé, briefing of healthcare providers on the use of the specific and prophylactic treatment for Ebola Virus Disease (EVD), and obtaining informed consent from positive patients with the INRB team for the addition of a new drug.

1.8. Risk communication and community engagement

1.8.1. Main actions

- In Ituri, 14,833 home visits reached 50,031 people and 746 educational talks reached 13,640 people; 46 mass awareness campaigns reached 6,553 people, 2 community debates reached 66 people, and 7 advocacy campaigns reached 29 people; 172 community alerts were reported (including 20 deaths), 7 of the 18 feedback points were resolved within 48 hours, and the 2 recorded community incidents were resolved in Nizi and Nia-Nia; in support of the pillars, 181 contacts were pre-listed around 13 confirmed cases, decontamination of 46 households and one healthcare facility was facilitated, 10/15 EDS (67%) were completed, and 14 positive results were announced
- In North Kivu, 41,093 people were reached during 17,000 home visits, 2,341 people were reached during 68 educational talks, 3,996 during 23 mass communication actions and 88 during 2 briefings; 174 community alerts notified out of 450 expected (38.6%), 7 feedbacks resolved out of 13 (53.8%), 1,186 people engaged out of 4,750 (24.9%) and 3 community incidents resolved (33.3%).
- In Bas-Uélé, 2,469 people were sensitized in schools (909 men and 1,560 women); briefing and advocacy with 19 priests of the diocese of Buta, sensitization of 10 members of the COGE of the Ganga health zone and support for the briefing of 39 healthcare providers in Buta.
- In Tshopo, 388 households visited reached 2,139 people, 2 educational talks reached 44 people in Isangi and mass awareness actions reached 1,871 people in the 5 health zones of Kisangani and 6 schools; 8 spots broadcast in 4 press outlets; 13 community alerts notified and all resolved, as well as 3 feedbacks and 1 community incident resolved within 48 hours.
- In Haut-Uélé, 43 educational talks reached 587 people and 254 home visits reached 1,285 people in the health zones of Wamba and Pawa; 4 mass actions reached 411 people at the Wamba CTE and 1 community dialogue reached 61 people in Isiro, while the dialogue conducted around the latest confirmed cases reached 59 people; 15 alerts of suspected cases and deaths were notified by community actors, 18 feedback issues were resolved within 48 hours, 2,344 people were engaged and 1 resistance incident was resolved in Isiro; the joint preparation for the release of 17 victors continues in Pawa, Wamba and Gombari.

1.8.2. Challenges

Persistent community resistance: In Ituri, repeated cases of resistance to the EDS (Emergency Health Diagnosis) program are recorded in Nizi, compounded by insufficient resources to support other pillars (mobility, a broken printer, and no internet connection). In North Kivu, there is demotivation among CREC (Community Health Center) staff who have not been paid for over three months, non-alignment of some partners with provincial planning, and numerous refusals, resistance, and sometimes violent community incidents. In Tshopo, there is a threat against a RECO (Community Health Center) that reported a community death, and a confirmed case who escaped from the CTE (Emergency Treatment Center) is still at home. In Bas-Uélé, there is a lack of transportation and demotivation among...

CREC actors, low reporting from health zones and impassable roads.

1.9. Mental Health and Psychosocial Support (MHPSS)

1.9.1. Main actions

- **In Ituri**, 15 people among the new confirmed cases and 103 of the 151 confirmed cases under follow-up (68%) benefited from SMSPS interventions, as well as 40 of the 116 new suspects (34%) and 104 of the 137 suspects under follow-up (76%); 56 results announced, including 15 positive cases, 67 PPLs supported and 39 households assisted, 25 children monitored, 6/6 children monitored in daycare and 12 of the 16 recovered (75%) monitored in the community.
- **In Tshopo**, 1 new confirmed case, 2 confirmed cases under follow-up, 3 new suspected cases and 6 suspected cases under follow-up benefited from SMSPS interventions; 5 results announced, including 1 positive; 13 accompanying persons at the CTE, 5 PPL and 25 contacts supported at the Makiso-Kisangani General Referral Hospital, psychological preparation of the EDS team and 2 recovered individuals integrated into the response teams.

1.9.2. In Haut-Uélé, 6 new confirmed cases and 24 confirmed cases under follow-up, 5 new suspected cases and 8 suspected cases under follow-up benefited from SMSPS interventions, as well as 70 accompanying persons at the CTE and 35 PPL; 28 laboratory results were announced, including 10 positive ones; 4 recovered (3 in Isiro and 1 in Wamba) were discharged without unloading kits.

1.9.3. Challenges

- **In Ituri**, there is low coverage of SMSPS interventions around the latest confirmed cases, with only 5 of the 9 notified health zones covered, and a need to strengthen collaboration with other pillars in Bunia and Komanda. In Tshopo, there is a lack of SMSPS providers in Wanie-Rukula, Bafwasende, and Isangi, and low partner participation in pillar activities.

1.10. Logistics

1.10.1. Main actions

- **In Ituri**, 26 patients were evacuated and 17 of the 33 organized transfers were carried out (51.5%). In North Kivu, 19 calls were received, resulting in 16 successful trips (96%), and 9 ambulances were mobilized out of 11 available (82%). Three calls could not be answered due to bed saturation at Katwa Hospital.
- **In Tshopo**, 2 calls were received for 2 trips carried out and all successful (100%), with 3 ambulances used out of the 3 available; monitoring of the construction of the Bafwasende CTE and the isolation of Madula, continuation of land procedures for the Makiso-Kisangani HGR, support for the mobility of pillars and provision of PCI inputs.
- **In Bas-Uélé**, no logistical data has been transmitted in the last 24 hours.

1.10.2. Security

1.10.3. Main actions

- Continued monitoring of the security situation.

1.11. PSEA (Prevention of Sexual Exploitation and Abuse)

1.11.1. Main actions. • In Ituri,

12 response staff members (7 men and 5 women) received the PSEA briefing; 385 people (236 men and 149 women) were sensitized in the community and two partners (WHO and Mercy Corps) reported on their PSEA activities.

In Bas-Uélé, a community consultation with 65 participants was held in Koteli, and an 11-member complaints management committee, including 9 women, was established.

1.11.2. Challenges

- Still insufficient coverage of PSEA activities, with only 13 of the 36 health zones in Ituri (36.1%) aware of the reporting mechanism; lack of mapping of stakeholders and refusal of some partners to align; insufficient mass awareness materials; lack of computer kits and office supplies for the pillar.

KEY MESSAGES

- Performance remains strong in epidemiological investigations (**100% of suspected cases investigated**) as well as an increase in the daily proportion of contact tracing (**88.0%**) similar to the monitoring (87.5%) which remains above the threshold of 85.0%.
- Sustained transmission is noted in the provinces of Ituri, North Kivu and Haut-Uélé, while the provinces of Tshopo and Bas-Uélé notify in a sporadic.
- In the last 24 hours, **87 new confirmed cases**, including **28 community deaths and 10 intra-CTE deaths**, have been reported, bringing the national total to **7,200 confirmed cases and 3,475 deaths (case fatality rate: 48.3%)**.
- Ituri remains the epicenter of the epidemic, accounting for **78.0%** of cumulative confirmed cases and **50.6%** of new cases.
- Strengthening priority interventions (surveillance, care, IPC, logistics and community engagement) remains essential to quickly interrupt transmission chains: Rapidly increase reception capacities in CT/CTE in North Kivu.
- Strengthen preparedness and response to Ebola Virus Disease-B in the river corridor provinces and those at risk.

TIMELINE OF KEY EVENTS

- From April 24 to May 15, 2026, the epidemic progressed from the detection of the first suspected cases to the biological confirmation of the Ebola Bundibugyo virus, leading to the official declaration of the 17th Ebola Virus Disease epidemic in the Democratic Republic of Congo.
- Between May 17 and 18, 2026, national authorities, with the support of partners, strengthened the response by declaring a public health emergency of national and then continental scope, accompanied by the mobilization of funds.



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